VITALITY 01

Sildenafil

Small-molecule PDE5 inhibitor taken by mouth; active ingredient in Viagra and Revatio



CLASS	STUDIED FOR	HUMAN EVIDENCE	US STATUS
PDE5 inhibitor	Erectile dysfunction, PAH (lungs)	Large trials; FDA approved	FDA approved (Viagra, Revatio)

STUDIED FOR

Erections	Lung artery pressure
HUMAN	HUMAN

Compounded forms are off-label. No published human study tested sildenafil as a gum, gummy or troche, or with oxytocin. Vybrique is the approved oral film.

HOW IT WORKS IN THE BODY

Sildenafil ▶ PDE5 blocked ▶ More cGMP ▶ Muscle relaxes

- **Blocks PDE5,** the enzyme that breaks down cGMP, per the label
- **Nitric oxide** released during sexual stimulation raises cGMP
- **No effect without** sexual stimulation, per the label

EVIDENCE & WHAT'S KNOWN

EVIDENCE

FDA APPROVED
LEVEL 5 OF 5

- Approved for ED in 1998, for PAH in 2005
- 21 placebo-controlled trials, 3,000+ men
- PAH trial (SUPER-1): 278 people, 12 weeks

NOT YET ESTABLISHED

- Combining with other PDE5 drugs
- Use after a recent heart attack
- Compounded gums and troches

QUICK PRACTICAL INFO

HALF-LIFE	ROUTES STUDIED
About 4 hours (drug and main metabolite)	By mouth (tablets, oral film); vein (Revatio)
PHARMACOKINETICS	**SOLD AS**
Peak in 30 to 120 minutes (median 60); liver (CYP3A4, CYP2C9)	Prescription tablets and film (Viagra, Revatio, Vybrique)
MONITORED IN TRIALS	**SPORT (WADA)**
Blood pressure, vision and hearing changes, erections over 4 hours	Not on the 2026 list

REGULATORY

FDA	Approved: Viagra 1998, Revatio 2005, Vybrique 2025
MHRA	Viagra Connect 50 mg: pharmacy medicine (2017)
EMA	Viagra authorised 1998; Revatio 2005
WADA	Not on the 2026 list

REFERENCES

1. FDA, Viagra prescribing information (DailyMed), Feb 2026
2. Goldstein et al., N Engl J Med, oral sildenafil for ED, May 1998
3. WADA, 2026 Prohibited List

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